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Guest Commentary | The push to outsource Santa Cruz County Public Health

By SANTA CRUZ SENTINEL
PUBLISHED: August 30, 2025 at 5:00 PM PDT

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Santa Cruz County is moving to end core public health clinic services, starting with the closure or outsourcing of in-house X-ray and laboratory functions; these diagnostics are essential for uninsured patients and enable rapid response to outbreaks. Administrators are pursuing this change under the promise of $2 million in savings — a short-sighted plan that may destabilize clinic health leading to a 7.5% drop in patient volume (just 1,065 people according to county estimates) would wipe out projected savings.

As public health guardians, we see a better path: expanding clinical services and investment in frontline staff to sustain public health services while generating revenue to benefit our residents and system.

For over 50 years, our county clinics served as a lifeline for roughly 14,195 vulnerable patients last year. Sixty-eight percent rely on our essential in-house lab and radiology services for timely diagnoses and treatment and the demand for these services has surged. Lab and radiology appointments are up 58% since 2022, and clinic visits are still rising.

Santa Cruz and Watsonville laboratories and radiology complete 50,000 blood draws and process 2,500 X-rays annually; the Integrated Community Health Center Commission has warned that outsourcing will reduce convenient options for patients. What is clear is that the county's Specialty Mental Health and Public Health Services rely heavily on Clinics Labs. Without the highly accessible and expert Clinics Labs services available an untold number of county residents with barriers to care will be unable to obtain care for their psychiatric crises, communicable illnesses or for opioid use disorder. Along with the lives lost and a preventable public health tragedy, the county may see dramatically increased costs as a direct consequence. Replacing trained union staff with low-wage, non-union workers is also not the answer, nor does this budget cut attenuate HSA's tone deafness in local governance. Non-union lab phlebotomists earning just $25.30 per hour, far below the local living wage, will likely destabilize patient care and lower local labor pay standards. External providers are not accountable to the public, the county, nor the Board of Supervisors.

At a time when public demand for integrated services reaches an all-time high, the evidence points to hiring more providers — not dismantling proven

At the June public hearing, the Board of Supervisors unanimously directed the Human Services Agency (HSA) to collaborate with frontline staff to develop alternatives to these cuts. Our County Clinics Frontline Innovation Plan does just that: keeping services in-house, expanding preventative care, increasing referrals and extending lab contracts to Juvenile Hall and the County Jail for example. In FY 2023-24 alone, County Clinics generated a $1.96 million surplus — showing that investment in frontline workers and services works.

The county blames their cuts on a revised Medicaid/MediCal reimbursement model, while stripping healthcare access from the most vulnerable. Yet, these cuts are a portent for a future of furious outsourcing and privatization. HSA will presume this is the appropriate pathway to cost savings regardless of health outcomes, wetting the appetites of businesses and their bottom lines for predictable surges in epidemic diseases and viral outbreaks.

The county had a clear opportunity to improve health outcomes, grow revenue and uphold public trust — but instead chose to trade it for uncertain short-term savings, a likely surge in preventable deaths and public health crises, weakened regulations and a model that prioritizes costs over lives. Even as HSA plans to steamroll community services, we will continue advocating for stronger care coordination, expanded in-house diagnostics and meaningful investment in our community.

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